

Tender and acute versus firm and chronic

A hordeolum is an acute infection; a chalazion is a sterile meibomian lipogranuloma.

Match the location and time course

Process	Distinguishing feature
External hordeolum	Tender lash-base pustule; Zeiss or Moll gland infection.
Internal hordeolum	Acute tender meibomian gland infection.
Chalazion	Slow firm relatively painless lump within the lid.
Posterior blepharitis	Capped meibomian openings and thick or turbid secretions.

Care for the lid margin

Warm compresses liquefy thick meibomian secretions; lid hygiene removes debris from the margin. A persistent uncomplicated chalazion may need incision and curettage. Do not label every persistent lump a chalazion: recurrence, lash loss, ulceration or disturbed lid architecture should prompt specialist assessment for malignancy.

A suspicious lesion needs a different pathway

Pearly edges, ulceration and lash loss are warning signs of lid malignancy.



Basal cell carcinoma phenotype: a lower-lid ulcer with a raised pearly rolled edge.

Distinguish the malignant patterns

Basal cell carcinoma commonly affects the lower lid and may have telangiectasia and central ulceration. Squamous cell carcinoma can show irregular everted, hyperkeratotic edges. Sebaceous gland carcinoma can mimic recurrent chalazion or chronic blepharitis. Persistent atypical lesions require specialist assessment and tissue diagnosis; the apparently benign label should be reconsidered when the clinical course does not fit.